A Nigerian Woman's Guide

What They Don't Tell You About PREGNANCY

Navigating All Three Trimesters with Confidence, Culture & Clarity

Written for every Nigerian woman growing a new life

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Navigating All Three Trimesters with Confidence, Culture & Clarity — From Lagos to Delivery Day

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A Note Before You Begin

This ebook is written from lived experience and extensive research. It is not a substitute for professional medical advice. Every pregnancy is unique — always consult your doctor, midwife, or healthcare provider for guidance specific to your situation.

That said, this guide was written to fill the gap between what doctors tell you and what nobody tells you. The things you wonder about at 2am. The things your aunty says that confuse you. The things that are completely normal but terrifying when they happen the first time.

You are not alone. Millions of Nigerian women have walked this path before you. This is their wisdom — and yours to keep.

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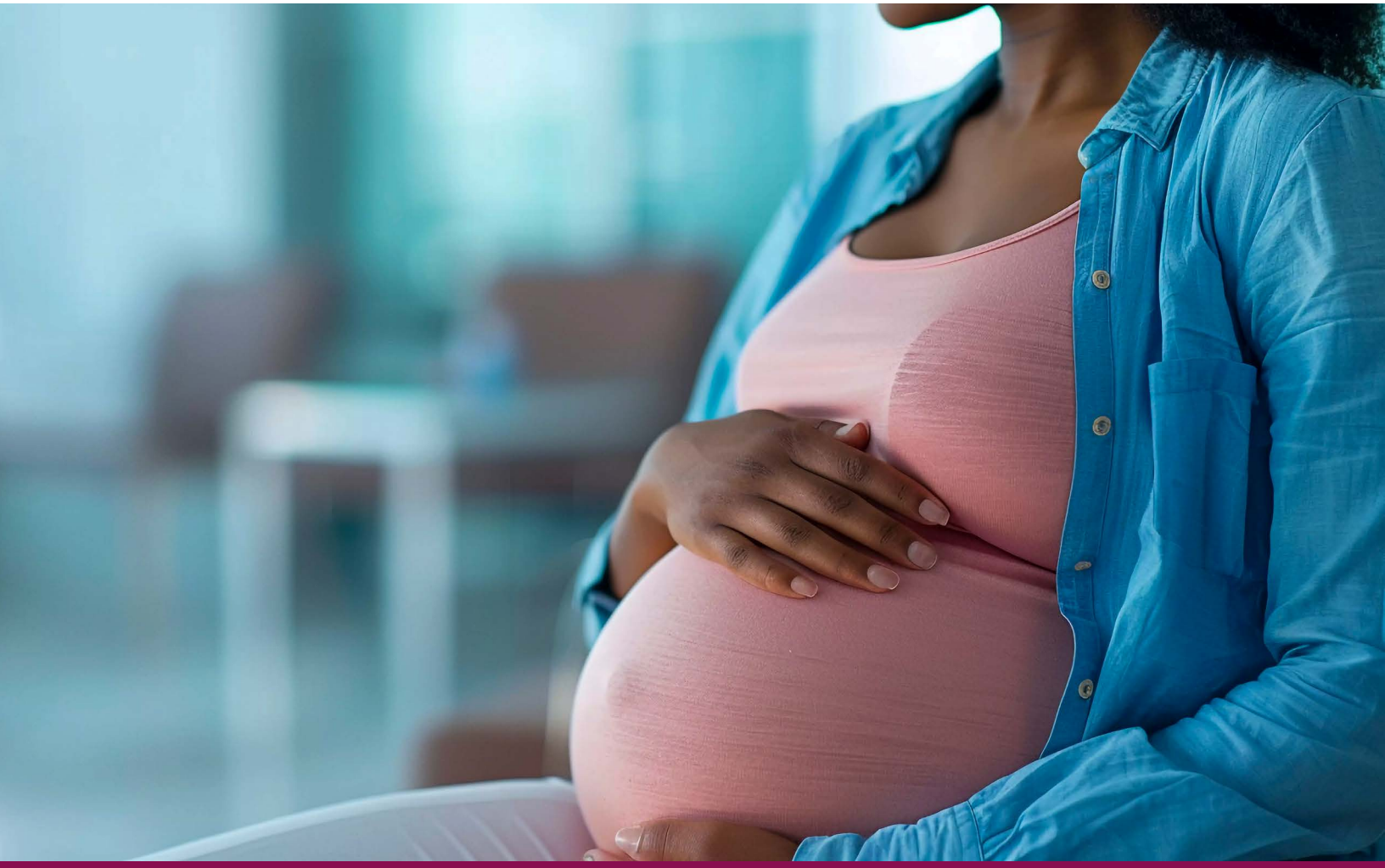

Introduction

Why This Guide Exists

When you find out you are pregnant, the first thing most Nigerian women do is Google everything. But quickly, you realise that most of the results are written for women in America or the UK. They talk about foods you cannot find in your local market, hospitals that work differently from ours, and cultural dynamics that have nothing to do with your reality in Lagos, Abuja, Port Harcourt, or your hometown.

This guide was born from that frustration — and from the experience of navigating pregnancy alongside a partner, discovering things the hard way, and wishing someone had just told us.

It is written for you: the Nigerian woman who is pregnant, excited, terrified, bloated, hopeful, and full of questions. You deserve a guide that speaks your language — not just English, but your cultural language.

How to Use This Book

You can read this from start to finish, or jump to the trimester you are currently in. Each section is designed to stand alone. At the end of key sections, you will find:

- Quick Tips — practical things you can do today
- What Your Partner Should Know — advice for the person supporting you
- Ask Your Doctor — questions worth raising at your next appointment

Bookmark the sections that feel most relevant right now. Come back to the others as your pregnancy progresses.

PART ONE

First Trimester

Weeks 1 – 13

What’s Actually Happening Inside You

Before you even know you are pregnant, your body has already begun an extraordinary process. In the first few weeks, a fertilised egg travels to your uterus, implants itself in the uterine wall, and begins dividing at a breathtaking pace. By week 6, a tiny heartbeat is flickering. By week 12, your baby has fingers, toes, and is beginning to look unmistakably human.

Meanwhile, your body is flooded with hormones — primarily hCG (human chorionic gonadotropin), progesterone, and oestrogen. These hormones are responsible for most of what you are feeling in the first trimester. They are doing their job. That is actually good news, even when it does not feel like it.

The Symptoms Nobody Warned You About

Everyone knows about morning sickness. But here is what they do not always mention:

1. Morning Sickness Is Not Just in the Morning

Despite its name, nausea can strike at any time — morning, afternoon, evening, or all day. Some women feel sick at the smell of certain foods (pepper soup, fried fish, and palm oil are common culprits). This is completely normal. It usually peaks around week 8–10 and eases by week 14 for most women.

> **Quick Tip:** Keep dry crackers or plain biscuits by your bedside. Eating something small before you get up in the morning can help settle your stomach before you face the day.

2. Extreme Fatigue

You may feel exhausted in a way you have never experienced before. Growing a human being is serious work. Your body is building a placenta from scratch, increasing blood volume, and managing a hormonal surge. Rest when you can. This is not laziness — it is biology.

3. Tender, Swollen Breasts

Your breasts will likely feel sore, heavy, and sensitive — sometimes to the point where even your clothing feels uncomfortable. This is your body preparing for breastfeeding. A good supportive bra makes a significant difference.

4. Frequent Urination

You may find yourself needing to urinate more often, even in the early weeks before your uterus is large enough to press on your bladder. This is caused by increased blood flow to the kidneys and hormonal changes. Stay hydrated — do not reduce water intake to avoid this.

5. Mood Swings

One moment you are fine, the next you are crying over a TV advert. Hormonal fluctuations affect your brain chemistry. Mood swings in the first trimester are normal. Communicating with your partner about what you are experiencing makes a significant difference.

6. Heightened Sense of Smell

Many pregnant women develop a super-powered sense of smell. Foods, perfumes, body odours — things that never bothered you before can suddenly make you nauseous. This is thought to be a protective mechanism to steer you away from potentially harmful substances.

Nutrition: Eating Well with Nigerian Foods

You do not need to abandon Nigerian food during pregnancy. In fact, many traditional Nigerian foods are excellent for pregnancy. The key is balance, variety, and knowing which specific foods to be cautious about.

Excellent Nigerian Foods for Pregnancy

- Beans (ewa) — rich in iron, protein, and folate; great for baby's neural development
- Ugwu (fluted pumpkin leaves) — excellent source of iron and folic acid
- Oats and whole grains — steady energy, good for managing nausea
- Eggs — complete protein, choline for brain development
- Fish (especially sardines, mackerel) — omega-3 fatty acids for brain and eye development
- Bitter leaf soup — good source of folic acid
- Plantain — provides energy, potassium, and vitamin B6
- Soybeans — excellent plant-based protein and calcium

Foods to Be Careful About

- Raw or undercooked meat — risk of listeria and toxoplasmosis
- Soft, unpasteurised cheese — not common in Nigeria but worth knowing
- Excessive pepper — can worsen heartburn and indigestion in some women
- Too much liver — high in vitamin A, which in excess can be harmful to baby
- Pawpaw (unripe) — unripe pawpaw contains latex-like substances that may trigger uterine contractions; fully ripe pawpaw is fine

Ask Your Doctor: Request a prescription for prenatal vitamins, especially folic acid (ideally started before conception and continued through the first trimester), iron, and calcium. Many Nigerian women are iron-deficient during pregnancy.

Navigating Healthcare in Nigeria

One of the biggest challenges of pregnancy in Nigeria is the healthcare system itself. Here is what you need to know:

Private vs. Public Hospital

If you have the means, many Nigerian women prefer private hospitals or private wings of government hospitals for prenatal care. They tend to offer shorter wait times, more attentive care, and better-equipped facilities. However, federal teaching hospitals have highly qualified specialists and are important for high-risk pregnancies.

Register Early

Register for antenatal care as soon as possible — ideally before 12 weeks. Early registration means your baseline health is documented, you receive important early scans, and any risk factors are identified promptly.

Key Antenatal Visits in the First Trimester

- Booking visit (first appointment) — blood tests, urine test, blood pressure, weight
- Dating scan (around weeks 8–12) — confirms how far along you are
- First trimester screening (if offered) — checks for chromosomal conditions

> **Quick Tip:** Keep a dedicated pregnancy folder or phone note with all your test results, scan images, appointment dates, and medication history. Nigerian hospitals sometimes lose patient files — having your own record protects you.

Emotional Rollercoaster: It Is Real

The emotional experience of early pregnancy is often overlooked. Many women describe feeling a confusing mix of joy, fear, guilt, anxiety, and overwhelm — sometimes all in the same hour. This is normal.

Fears are common: Will I miscarry? Is the baby healthy? Will I be a good mother? These fears do not mean something is wrong. They mean you care deeply. However, if anxiety is severe or persistent, speak with your doctor. Antenatal anxiety and depression are real conditions that deserve care and attention.

"The bravest thing I did during my first trimester was admit to my husband that I was scared. Once I said it out loud, it lost some of its power."

PART TWO

Second Trimester

Weeks 14 – 27

The ‘Honeymoon Phase’ — and Why It Is Not Always That

The second trimester is often called the honeymoon phase of pregnancy. For many women, the nausea fades, energy returns, and the bump becomes visible in a beautiful way. But not every woman experiences this. Some continue to feel unwell. Some develop new symptoms. And many feel a mix of relief and new anxieties.

Whatever your experience, you are not doing it wrong. Pregnancy does not follow a script.

Your Body Is Changing Visibly

The Baby Bump

By weeks 16–20, most women will have a visible bump. First-time mothers may show later; women who have been pregnant before often show earlier. Do not compare your bump to anyone else’s. Bump size depends on your height, weight, baby’s position, and many other factors.

Round Ligament Pain

Sharp, shooting pains on one or both sides of your lower abdomen are very common in the second trimester. This is round ligament pain — caused by the ligaments that support your uterus stretching as your uterus grows. It is harmless and usually brief. However, if the pain is severe or persistent, always consult your doctor.

Skin Changes

Many Nigerian women notice hyperpigmentation during pregnancy — darkening of the skin on the face (called melasma or 'the mask of pregnancy'), a dark line down the centre of the abdomen (linea nigra), and darkening of the areolas. These are caused by increased melanin production and are temporary for most women.

Back Pain

As your centre of gravity shifts, back pain becomes increasingly common. Wearing supportive shoes, sleeping with a pillow between your knees, and gentle stretching or prenatal yoga can help significantly.

Prenatal Care: Tests, Scans & What They Mean

The Anomaly Scan (Weeks 18–22)

This is one of the most important scans of your pregnancy. The sonographer will carefully examine your baby's organs, limbs, face, and spine to check for any abnormalities. They will also check the placenta and amniotic fluid. This scan is usually when parents choose to find out the sex of the baby — though in some Nigerian hospitals, staff may be reluctant to disclose this.

Glucose Tolerance Test (Weeks 24–28)

This test checks for gestational diabetes — a form of diabetes that develops during pregnancy. You will be asked to drink a glucose solution and have your blood tested. Gestational diabetes is manageable, but if undetected, it can cause complications. Women who are overweight, have a family history of diabetes, or had gestational diabetes in a previous pregnancy are at higher risk.

Ask Your Doctor: If you are between weeks 18–22, ask specifically about the anomaly scan. Not all Nigerian hospitals offer it as a standard procedure — you may need to request it or visit a specialist imaging centre.

Common Myths in Nigerian Pregnancy Culture

Nigerian culture is rich with pregnancy traditions, beliefs, and advice — some of which are genuinely helpful, and others that can cause unnecessary worry or even harm. Here are some of the most common myths, addressed honestly:

Myth: You must eat for two

Reality: You only need about 300–500 extra calories per day during pregnancy — roughly equivalent to an extra bowl of oats or a banana and a handful of nuts. Overeating during pregnancy increases the risk of gestational diabetes and excessive weight gain. Eat more nutritiously, not necessarily more in quantity.

Myth: Certain foods determine the baby's complexion

Reality: Your baby's skin colour is determined entirely by genetics — the combination of genes from both parents. No amount of milk, tomatoes, or any other food will change this. However, good nutrition absolutely affects your baby's health and development.

Myth: You should not exercise during pregnancy

Reality: For most women with uncomplicated pregnancies, moderate exercise is not only safe but highly beneficial. It reduces back pain, improves mood, helps with sleep, and may even lead to shorter labour. Walking, swimming, and prenatal yoga are excellent options. Always get your doctor's clearance before starting or continuing an exercise routine.

Myth: Going out at night is dangerous for the baby

Reality: There is no medical basis for this belief. However, it is worth noting that fatigue is real in pregnancy, and common sense safety precautions — good lighting, safe transport, not overexerting yourself — are always wise.

Preparing Your Home and Budget

The second trimester is the ideal time to begin practical preparations — you are past the fragility of the first trimester but not yet weighed down by the discomforts of the third.

Financial Planning

- Research hospital delivery costs — prices vary enormously between facilities
- Investigate your health insurance coverage, if applicable
- Begin setting aside a baby emergency fund
- Budget for baby essentials: cot, car seat, feeding supplies, clothing
- Factor in maternity leave — understand your employer's policy

Home Preparation

- Begin decluttering and creating space for the baby
- Research paediatricians in your area — register before delivery if possible
- If you have older children, begin talking to them about the new baby

PART THREE

Third Trimester

Weeks 28 – 40+

The Final Stretch

The third trimester is a time of profound physical change and emotional intensity. Your baby is growing rapidly — gaining about half a pound per week in the final weeks. You may feel heavy, uncomfortable, and impatient. You may also feel a deep, quiet awe as you count down the days.

Physical symptoms in the third trimester often include:

- Shortness of breath — your uterus is pressing on your diaphragm
- Heartburn — the same pressure affects your stomach
- Swollen ankles and feet — increased fluid retention
- Braxton Hicks contractions — practice contractions; tightening of the uterus
- Difficulty sleeping — finding a comfortable position becomes an art form
- Frequent urination — your bladder has very little room now
- Pelvic pressure — the baby's head may be descending into your pelvis

Quick Tip: Sleep on your left side in the third trimester. This improves blood flow to your baby, your kidneys, and your heart, and reduces pressure on your liver. Use a pregnancy pillow or fold a regular pillow between your knees and under your bump.

Signs of Labour: Real vs. False

Braxton Hicks vs. Real Contractions

Braxton Hicks contractions are irregular, usually painless, and do not increase in frequency or intensity. Real labour contractions are regular, progressively stronger, longer, and closer together. A helpful rule of thumb: if the contractions follow the 5-1-1 pattern (every 5 minutes, lasting 1 minute, for 1 hour), it is time to go to the hospital.

Other Signs That Labour May Be Starting

- Your 'mucus plug' passes — a thick, jelly-like discharge, sometimes tinged pink or brown
- Your waters break — a gush or trickle of clear or slightly pink fluid
- Strong, regular contractions that do not ease with rest or movement
- Lower back pain that comes in waves, accompanied by abdominal tightening

Go to the hospital immediately if: Your waters break and the fluid is green, brown, or foul-smelling; you have heavy bleeding; you feel the umbilical cord or see it; your baby stops moving significantly; or you have a severe headache with vision changes (sign of pre-eclampsia).

Hospital Bag Checklist for Nigerian Mothers

For You

- Valid ID and all antenatal records and scan images
- Health insurance card (if applicable) or cash for hospital fees
- Comfortable nightgowns or wrappers (front-opening for breastfeeding)
- Slippers or flip-flops for the ward
- Toiletries: soap, toothbrush, deodorant, skincare
- Sanitary towels or maternity pads (heavy flow — bring more than you think)
- Change of clothes for going home
- Snacks for labour — energy bars, crackers, juice
- Phone and charger

For Your Baby

- 3–5 baby vests and sleepsuits (newborn and 0-3 months sizes)
- Baby mittens and socks
- Soft towels or receiving blankets
- Nappies (newborn size, bring at least 20)
- Baby wipes (unscented)
- Cotton wool for cleaning the umbilical stump
- Car seat (mandatory if driving home)

For Your Partner / Support Person

- Change of clothes and toiletries
- Cash for hospital incidentals
- Phone charger
- Snacks and drinks

Understanding Your Birth Options in Nigeria

Vaginal Birth

Vaginal birth is the most common form of delivery in Nigeria and, for most women with uncomplicated pregnancies, it is the recommended option. Recovery is generally faster than after a caesarean section. Pain relief options vary by facility — discuss what is available at your hospital in advance.

Caesarean Section (C-Section)

A C-section may be planned in advance (elective) or performed as an emergency. Reasons for a C-section include: baby in breech position, placenta previa, pre-eclampsia, prolonged labour, or foetal distress. A C-section is major surgery — recovery takes 6–8 weeks. Do not feel any shame or guilt if you have one. The goal is a healthy baby and a healthy mother.

Postpartum: What Happens After Delivery

The immediate postpartum period — the first six weeks after delivery — is one of the least discussed and most challenging phases. Here is what to expect:

Physical Recovery

- Lochia — postpartum vaginal bleeding that lasts 4–6 weeks, gradually reducing
- Uterine cramps as your uterus contracts back to size
- Perineal soreness if you had a vaginal birth (especially if you had stitches)
- Breast engorgement as your milk comes in (usually 2–4 days after delivery)
- Night sweats — your body shedding excess fluid from pregnancy

Emotional Recovery

Baby blues — tearfulness, mood swings, and emotional sensitivity in the first two weeks — affect up to 80% of new mothers and are caused by the dramatic hormonal shift after delivery. They typically resolve on their own.

Postpartum depression is different — it is deeper, lasts longer, and may include persistent sadness, anxiety, inability to bond with your baby, or thoughts of harming yourself. If you experience these symptoms, please speak to your doctor immediately. Postpartum depression is a medical condition, not a personal failure.

Bonus: Partner's Survival Guide

If you are a man reading this — well done. The fact that you are here says a great deal about the kind of partner and father you intend to be. Here is what your pregnant partner needs from you:

What She Needs Most

- Presence — show up, physically and emotionally
- Patience — hormones are real; do not take mood swings personally
- Practical help — cook a meal, clean the house, carry the groceries
- Advocacy — speak up for her at hospital appointments when she is too tired
- Reassurance — tell her she is doing an incredible job, often

What Not to Do

- Do not compare her pregnancy to others ('My sister was fine during hers...')
- Do not dismiss her symptoms as exaggerated
- Do not leave all decision-making to her — this is your child too
- Do not disappear into work and assume everything is fine

> **For Couples:** Consider attending at least one or two antenatal appointments together. It deepens your connection to the pregnancy and helps you both ask questions and understand what is happening.

Bonus: Navigating Family & Unsolicited Advice

In Nigerian culture, pregnancy is a community event. Your mother, mother-in-law, aunties, neighbours, and random women at the market will all have something to say. Some of it will be wonderful. Some of it will drive you to the edge of sanity.

How to Handle Unsolicited Advice Gracefully

- Thank people for their concern before redirecting: ‘Thank you, I’ll mention that to my doctor’
- Pick your battles — not every piece of advice needs a response or argument
- Have a united front with your partner on key decisions so you are not pulled in different directions
- Set gentle but firm boundaries around your birth plan, feeding choices, and postpartum support

The people giving you advice usually mean well. They are drawing on their own experiences and cultural knowledge. You can receive their love while still making your own informed decisions.

Quick Reference: When to Call Your Doctor

Always trust your instincts. If something feels wrong, call. But here are the specific signs that warrant immediate medical attention at any stage of pregnancy:

- Heavy vaginal bleeding at any point
- Severe abdominal pain or cramping
- Baby stops moving, or you notice a significant reduction in movement after week 28
- Severe headache, blurred vision, or sudden swelling of hands and face (signs of pre-eclampsia)
- Fever above 38°C
- Burning or pain when urinating (possible UTI, which can trigger preterm labour if untreated)
- Signs of your water breaking before week 37
- Chest pain or difficulty breathing
- Any fall or trauma to your abdomen

Important: Save your doctor's emergency number, your hospital's number, and the number of a trusted driver in your phone now — before you need them. Do not wait until you are in pain to search for these contacts.

A Final Word

Pregnancy is one of the most extraordinary things a human body can do. It is also one of the most challenging, confusing, and emotionally complex experiences you will ever have. Both things are true at the same time.

You will have days when you feel like a goddess — glowing, powerful, and in awe of what your body is doing. You will also have days when you are exhausted, scared, and ready for it all to be over. Both days are valid.

The goal of this guide was never to make pregnancy seem easy. It was to make it seem less unknown. Because the things that frighten us most are usually the things nobody warned us about.

Now you know. And knowledge — as every Nigerian mother will tell you — is power.

Congratulations. You are going to be wonderful.

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